



Mental Health Inpatient Triage to Discharge SOP

Scope

Site	Department, division, or operational area	Applicable to
Royal Perth Bentley Group (RPBG)	Mental Health Division (MHD) All inpatient areas (Excluding General Health areas)	All staff

Purpose

This standard operational procedure (SOP) provides guidance for admission into a mental health (MH) inpatient unit, ongoing care, transfer, and discharge processes.

Contents

Definitions and abbreviations.....	2
Procedure	4
Principles	4
Referral pathways.....	5
Admissions	5
Reviews and ongoing care.....	6
Specific care needs	6
Escalation of care processes.....	7
Transfer of care	7
Inter-ward transfers within RPBG sites.....	7
Interstate transfers.....	9
IHPTs: Metropolitan transfers external to and from RPBG	9
Discharge.....	10
Escort and transport considerations	10
Unplanned transfer	11
Compliance monitoring	11
Appendix I: Mental Health (MH) Direct and Emergency Presentations to RPBG.....	15
Chart 1 – RPBG presentation and management of Mental Health patients.....	15
Chart 2 – Referral Pathway from RPH ED to RPBG Inpatient MH Services.....	16
Appendix II: Specific ward/unit requirements	17
Appendix III: MHEC Admission.....	18
MHEC Traffic Light System, RPH.....	18
Appendix IV: Specific Ward 2K requirements (RPH)	20
Appendix V: Ward 10 (BHS) - Older Adult (OA) specific requirements.....	21
Appendix VI: Transitional Care Unit (TCU) specific requirements (offsite).....	22
Appendix VII: MH Division Inpatient Ward Security Checklist.....	24

Note: For this document personal support persons (PSPs) includes next of kin (NoK), family, carers, an Office of the Public Advocate guardian or enduring guardian or a nominated person and will be referred to as “PSP.”

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Definitions and abbreviations

BHS	Bentley Health Service
CMHS	Community Mental Health Services
C-L	A Consultation Liaison Psychiatry Service based at RPH, consisting of MDT of Consultant Psychiatrists, Addiction Medicine Consultant, Training Registrars, Specialist Nurses, and Clinical Psychologists who provide assessment, diagnosis, and management of patients with MH illness or concerns in non-MH settings across RPH.
CP	Chief Psychiatrist, Western Australia
CTS*	<i>Care Transfer Summary</i>
EBM	Enterprise Bed Management
ED	Emergency Department
EDIS	ED Information System
EMHS	East Metropolitan Health Service
EMyU	East Metropolitan Youth Unit
EPiC	Emergency Physician in Charge
GP	General practitioner
HMC	Hospital Medication Chart
HSP	Health Service Provider
IHPT	Interhospital Patient Transfer
MDT	Multidisciplinary team
MH	Mental health
MHA 2014	Mental Health Act, 2014 , Western Australia
MHAF*	<i>Mental Health Assessment Form</i> including Mental State Assessment, also referred to as Mental State Examination (MSE)
MHEC	Mental Health Emergency Centre
MHERL	Mental Health Emergency Response Line
MHPF	Mental Health Patient Flow
NaCS	Notifications and Clinical Summaries
NFA	No fixed abode
NOCC	National Outcomes and Casemix Collection - a set of outcome measure (mandatory)
Nominated person	The person nominated by a patient under section 273(1) of the Mental Health Act 2014 (MHA 2014) and must be over the age of 18 to receive information and ensure the patient's rights and interests are upheld. ¹
OHC	Operations Hub Coordinator

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Definitions and abbreviations cont'd

PLS	Psychiatric Liaison Service, RPH ED
PRA	Progressive Risk Assessment
PSOLIS/WebPSOLIS	Psychiatric Services Online Information System – paper and web based
PDD	Planned Discharge date
RAMP *	<i>Risk Assessment Management Plan</i>
RPH	Royal Perth Hospital
SSCD	State-wide standardised clinical documentation (mandatory)
Suppression of medical record (previously Anonymity – Code 90)	A patient may request the admitting clinician or Treating team to record suppression of the medical record for their current inpatient admission until discharge or until the patient chooses to cancel the request. Consequently, no information will be released by the hospital or health service to any person (family or friend) regarding the patient's admission or hospital status, unless for e.g. • Ordered by a Court • Required by Police as part of a criminal investigation • Required in accordance with the Guardianship and Administration Act 1990 • Imminent risk of harm to self or others • Released as a Discharge Summary letter to patient's GP Note: in the case of an unforeseen event (e.g., an emergency or transfer to another hospital or service), RPBG reserves the right to inform the nominated person.
TCU	Transitional Care Unit (Bidi Wungen Kaat Centre).
TSDP*	Treatment, Support and Discharge Plan is a patient care plan.
WACHS	Western Australian Country Health Service
webPAS	Web-based Patient Administration System.

*SSCD form

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Procedure

- Clinical activities relating to the admission readmission discharge and transfer of patients in inpatient mental health services are in accordance with the requirements of the **MHA 2014**.

Principles

- Use a multidisciplinary approach, ensure care is aligned with the principles of recovery and trauma informed care as per the CP's Standards for Clinical Care and in accordance with the following, the
 - [Charter of Mental Health Principles - Office of the Chief Psychiatrist WA](#)
 - [WA Department of Health - National Framework for Recovery-Oriented Mental Health Services](#) ²
 - [WA Department of Health - Triage to Discharge Mental Health Framework for Statewide Standardised Clinical Documentation \(SSCD\)](#) which outlines required timeframes for completing key documentation and relevant screening tools
 - [RPBG Clinical Handover Policy](#)
 - East Metropolitan Health Service ([EMHS](#)) [Mental Health Patient Flow and Bed Capacity Framework Process](#)
 - [EMHS Care Coordination in Mental Health Services Policy](#)
 - RPBG Medical Record Transfer and Access SOP
 - [RPBG Inpatient Admissions from the Emergency Department Policy](#).
- Provide an integrated mental health service that connects emergency, inpatient and community services
- Support diversity including sensitivity to the needs of Aboriginal people and other cultures, diverse age, ethnicity, religion, and individuals with sexual, gender and/or bodily diversity
- Ensure emphasis on transition planning from the time of admission with a planned discharge date (PDD)
- Employ the least restrictive intervention necessary to ensure the safety of patient, staff, and others.

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Referral pathways

- For specific referral criteria, referral forms and service-specific models of care, and Policies, refer to [RPG Mental Health Division HealthPoint page](#)
- Primary referral pathways to inpatient units are:
 - RPH ED – refer to [Appendix I: Mental Health \(MH\) Direct and Emergency Presentations to RPG](#)
 - Direct admission which can occur to the ward or service if a bed is available and the patient meets ward or unit inclusion criteria, e.g.
 - CMHS, EMHS inpatient areas, residential or aged care facilities, courts, detention centres, RPG Outpatient services, etc.
 - Refer to [Appendix I: Mental Health \(MH\) Direct and Emergency Presentations to RPG](#)
- Referrals from other HSPs will be considered on a case-by-case basis.

Admissions

Allocated, admitting Inpatient MHS clinician to:

- Confirm with Shift Coordinator any roles and responsibilities for escort and transport as required. Refer to [Escort and transport considerations](#) section
- For admission of patients younger than 18 years, ensure completion of the [Chief Psychiatrist of WA MHA 2014 s.303 - Segregation of Children from Adult Inpatients - Notification Form](#) (MHA_s303)
- Notify the medical team (i.e., Senior Medical staff or Resident Medical Officer) of the expected patient arrival
- Provide the following information to patient and/or PSP
 - Welcome Pack (explain rights and responsibilities of patient and PSP expectations and document in patient integrated notes, recording the date information provided)
 - How to request medical reviews (Refer to [EMHS Rights of Patients in Mental Health Policy](#))
 - Access to Aboriginal Health Liaison Officer, asking Aboriginal patients if they would like significant members of their community, including elders and traditional healers, involved in their assessment, examination, and treatment
 - Other cultural supports, language services, LGBTIQ+ Allies, etc.
- Complete property checks and storage of personal property items
- Complete patient's identification (ID) and suppression of medical record, if required
- Obtain consent to planned treatment during admission as part of the clinical assessment and complete the *PSP and Consent to Release/Receive Information Form*
- Complete *Hospital Notification eForm* to GP or other health service
- Inform the patient, and PSP of expected ongoing care. Include other services (e.g., CMHS, detention centre) as relevant

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Referral pathways – Admissions cont'd

- Complete all SSCD in PSOLIS or WebPSOLIS or WebPAS and other admission requirements, including
 - NOCC measures
 - MH legal status and MHA 2014 forms, as appropriate
 - PSOLIS alerts, as relevant - clerical staff to print and file in medical record on pink form
 - Comprehensive care documentation within target timeframes
 - Other screening or assessment tools, (e.g., substance use, cognitive), if required
- Orientate patient to Inpatient unit, immediate care environment
- Ensure clerical staff have actioned activation via PSOLIS and WebPAS
- Complete intentional rounding and observations
- Complete vital signs as per [Vital Signs Nursing Practice Standard \(NPS\)](#)
- Additionally, TCU service staff are to practice specific requirements within Appendix VI: e.g., specific, leave, rounding and observation requirements
- Complete all details of [Appendix VII: MH Inpatient Security Checklist](#) at commencement of shift (minimum) and more frequently as required
- Commence early discharge planning
 - Medical staff to commence *Notifications and Clinical Summaries (NaCS)* discharge summary

Reviews and ongoing care

Clinical review is to occur as per [Minimum Requirement for Medical Review of Patients Admitted to Inpatient Wards/Community Mental Health Services Policy](#).

MDT to:

- Review and/or update the following – noting review dates and outcomes in patient integrated notes:
 - Multidisciplinary comprehensive care documentation suite
 - Relevant SSCD documentation
 - PRA as required - TCU refer to [Appendix VI: TCU specific requirements](#)
 - Other documentation, screening, and assessments within target timeframes
 - NOCC measures
- Maintain intentional rounding and observations
- Identify and make early efforts to source suitable crisis accommodation for patients with NFA.

Specific care needs

For patients in need of:

- Electroconvulsive therapy (ECT)
 - To be preferentially referred and admitted to Bentley Health Service (BHS). Refer to [ECT via Thymatron IV® Management CPS](#)
- Clopixol Acuphase (Zuclopenthixol) - refer to [Zuclopenthixol Acetate \(Clopixol Acuphase®\) Intramuscular Injection SOP](#)
- Lithium - refer to [Lithium Clinical Guideline](#)

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Reviews and ongoing care – Specific care needs cont'd

- Clozapine - refer to [Clozapine Prescribing, Administering, Monitoring and Capillary Sampling SOP](#)
- Olanzapine long-acting injection - refer to [Olanzapine Long-Acting Injections SOP \(Mental Health\)](#)
- Eating Disorders – refer to [Eating Disorders: Management of Patients SOP](#).

Escalation of care processes

Monitor patient's mental state and cognition and physical health. For guidance relating to the escalation of patient care:

- RPBG documents
 - [Recognising and Responding to Acute Deterioration \(RRAD\) in Mental State, Cognition and Behaviour SOP](#)
 - [RRAD Policy](#)
 - [SAFE and BeSAFE Policy](#). For TCU, refer to [Appendix VI: TCU specific requirements](#)
 - [Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP](#)
- For complex patients, management plans developed in collaboration with specialist teams outside of MH may be required.
- In circumstances in which children or dependents may be at risk or suspicion of abuse and/or neglect refer to [Social Work Response to Child Protection Concerns SOP](#) and [Recognising and Responding to Family and Domestic Violence and Elder Abuse SOP](#).

Transfer of care

Patients may require an inter-ward or Interhospital Patient Transfer (IHPT) to meet mental and physical health care requirements.

Inter-ward transfers within RPBG sites

(e.g., within BHS, RPH to BHS, BHS to RPH, TCU to RPH)

The MO of transferring unit to:

1. Liaise with the relevant receiving ward/unit MO or Consultant
 - Patient's treatment and care to be discussed and handover to be provided in iSoBAR format
 - Name and contact details of staff accepting transfer to be documented in integrated patient notes
 - Acceptance of care to be confirmed
 - MH legal status and MHA forms (as appropriate), including PSOLIS alerts and incidents
 - *Notifications and Clinical Summaries (NaCS)* and *WA Hospital Medication Chart (HMC)*
 - Inform patient and PSP of transfer plans and continue to update all parties as is practicable

Transfer of care – Inter-ward transfers within RPBG sites cont'd

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2. Allocated nurse or delegate to:

- Complete transfer request and book bed in EBM
 - For receiving hospitals or services without EBM, liaise with the ward bed manager
- Destination ward will then allocate patient to an available bed in EBM, indicating bed ready time
 - If acceptance of care is not received within one hour, and there is a bed available on destination ward, escalate to
 - In hours: **EMHS Mental Health Patient Flow (MHPF) Coordinator** 07:00–17:00 hrs, 7 days a week
 - After Hours (A/H): **ED Psychiatric Liaison Service** who will liaise with Operations Hub Coordinator (**OHC**) (17:00–07:00 hrs, 7 days a week) if required, i.e., if a bed becomes available.
- Email the following referral documentation (scanned) to destination ward or service, copying in EMHS MHPF or OHC or ED Psychiatric Liaison Service
 - New *RAMP* (SSCD)
 - *PRA* (except TCU)
 - *Mental Health Assessment Form (MHAF)* (SSCD)
 - Comprehensive Care documentation
 - *Care Transfer Summary (CTS)* (SSCD)
 - NOCC as appropriate
- Ensure bed is allocated by receiving ward or unit before patient is transferred
- Ensure completion of *Care Transfer Summary (CTS)* (SSCD) and notes of **transfer critical information**, note current mental state, physical and dental health needs (specifically addressing physical health care needs)
- Complete NOCC measures, as relevant
- Complete PSOLIS alerts, as relevant
- Update journey board and inform clerical staff to update WebPAS daily with estimated date of discharge and transfer information
- Consider the following requirements and action where applicable:
- **Medications on transfer and discharge**
 - Identification of relevant medications to be transferred with patient (e.g., patient's own medications [POMs] and/or prescribed medication for inpatient admission – consider refrigerated and S8/S4R medication) / equipment and/or consumables.
 - Doses needing to be administered prior to transfer or discharge
- **Patient** property arrangements
- **Patient** escort/transport arrangements – refer to [Escort and transport considerations](#) section.

Transfer of care – Inter-ward transfers within RPBG sites cont'd

General health ward: Direct admission or inter-ward or inter hospital planned transfer pathways or unplanned transfers (e.g., to RPH State Major Trauma Unit, Acute Medical Unit, Acute Surgical Unit, between RPH and BHS)

Refer to:

- [General Health Admission, Discharge and Transfer SOP](#)
- [Unplanned Transfers SOP \(BHS and TCU\)](#)
- [Unplanned Presentations to BHS SOP](#)
- [Elective Surgical Waitlist \(ESWL\) Management SOP](#) (planned or unplanned direct admission pathways)
- [Medical Record Transfer and Access SOP](#) for patients going to outpatient clinic or procedural units and returning to an inpatient mental health area.

Interstate transfers

Refer to [Escorting and Transferring Patients Intrastate, Interstate and Internationally Policy](#).

IHPTs: Metropolitan transfers external to and from RPBG

- All IHPTs are to be managed according to [EMHS Inter Hospital Patient Transfer Policy \(IHPT\)](#)
- If IHPT, Nursing staff to complete *IHPT Form* (retain copy for RPBG medical record)
- The unit Consultant /Medical Team to complete final clinical reviews prior to PDD, note date of discharge meeting in the integrated notes
- RPBG medical staff to liaise with destination medical staff to provide clinical handover
 - For transfers to country hospitals, contact [WACHS Link HealthPoint page](#) or call 1800 807 554
 - Complete the relevant referral form and email (preferred) to: WACHSLink@health.wa.gov.au (alternatively, fax 9223 8599)
 - MO to provide accepting hospital or Consultant name to Shift Coordinator
 - Medical staff to provide **NaCS and CTS**, MHA forms (as appropriate), *WA HMC*
- Shift Coordinator or Nurse Unit Manager (NUM) to
 - Create the IHPT or discharge details and record outcome (e.g., acceptance) into EBM and integrated notes. If refused, no EBM booking slip required
 - For receiving hospitals or services without EBM, liaise with the ward bed manager
- Complete *RAMP* and transfer *PRA* (except TCU) and consider transport needs
- Refer to [Escort and transport requirements](#) section
- Request for the collated, full medical record to accompany the patient at the time of transfer
- Escalate to OHC (RPH) or Site Director BHS or A/H, On Call Executive if wait time is greater than 48 hours
- Additionally, refer to [Discharge](#) section.

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Discharge

- Medical staff to complete *NaCS* and *CTS* and provide copy of *CTS* to patient and PSP
- Inform patient and PSP of discharge (or IHPT) and provide relevant information
- Clerical staff to update WebPAS with PDD and transfer or discharge information
- Update SSCD and other clinical documentation
- Consider use of the Discharge envelope
- Update PSOLIS - Activation/Deactivation
- For patient-initiated discharge (PID) and discharge against medical advice (DAMA), complete an alert on PSOLIS
- Ensure that the latest *MHAF* is carried forward to a new volume file, as appropriate
- Return patient property as per [Potentially Harmful Items Management for Mental Health Inpatients SOP](#)
- Post discharge, complete 7-day follow up for patients in PSOLIS Service event.

Escort and transport considerations

The Shift Coordinator, NUM, MO or DMO, from the ward or unit of origin to

- Ensure appropriate transportation requirements
- Check if patient is eligible for Patient Assisted Travel Scheme (PATs)
- Utilise computerised assisted radio personnel system (CARPS) or verbally to arrange transfer of patient return to original facility, ensure orderlies and Patient Care Assistants are aware
 - For BHS IHPT of **low and medium-acuity patients**, use the [Patient Transfer Coordination Hub](#) (PaTCH) booking system
- Determine level of clinical or non-clinical escort required and availability of appropriately skilled staff - including receiving service staffing or Shift Coordinator – particularly A/H
 - Where applicable, escort to request notification should patient require escort on return journey along with estimated collection time
 - Ensure other supports as per patient's risk factors, e.g., clear pathways, re-direct traffic, locking of specific entrances other patient/visitors or PSP management
 - Consider use of RPH Transit Lounge/unit activity spaces
- Ensure escorts have on their person a fully charged mobile with relevant numbers and a duress alarm
- Staff escorting the patient must:
 - Have the expertise appropriate to patient acuity or specific condition to provide the appropriate level of care
 - Be familiar with all equipment required during transfer
 - Be familiar with escalation process in event of patient deterioration enroute and means of communication.
- Escalate transfer priority if patient deteriorates prior to/during transfer and notify receiving site as soon as possible (via mobile phone). Non-hospital services, **telephone 000**

Escort and transport considerations cont'd

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- Record all adverse events (escort related) on Datix Clinical Incident Management System (CIMS)
- Refer to [Inter-Hospital Transfer Between St John Of God Midland Public and Private Hospital and RPH SOP](#) and [General Health Admission Discharge and Transfer SOP](#).

Unplanned transfer

- Refer to [Unplanned Transfer \(BHS and TCU\) SOP](#).

RFBG staff to use versions of the SSCD that are on WebPSOLIS. Use writeable PDFs for SSCD not available in PSOLIS. Hard copy documents are only to be used during downtime. Clinician, NUM, or delegate to transcribe information into PSOLIS within 24 hours. Refer to the [WA Department of Health Triage to Discharge Mental Health Framework for SSCD](#).

Compliance monitoring

The MHD Directors hold overall responsibility for ensuring compliance monitoring with this SOP, including the following:

- Audited as per the RFBG Performance Monitoring Schedule and reported by the MHD Clinical Quality and Safety Team on ward or service dashboards
- CIMS and/or Combined Hazard or Incident Reporting (CHoIR) (e.g., clinical incidents related to admission or treatment) and/or Office of the Chief Psychiatrist notifiable reports
- Compliments/Complaints Management System.

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Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Mental Health Inpatient Triage to Discharge SOP through consultation with EMHS Aboriginal Health Strategy Team and the Aboriginal Workforce Engagement Group to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 2535).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Community Mental Health Services Triage to Discharge SOP](#)
- [Language Services: Use of Interpreters Policy](#)
- [Identifications of Patients SOP](#)
- [Respiratory Virus Infection Prevention and Management Clinical Guideline](#)
- [Leave for Mental Health Inpatients Policy](#)
- [Medication Supply to Non-Admitted or Discharged Patient by Prescribers SOP](#)
- [Ligature Risks Management, Safe Use and Storage of Ligature Cutters Policy](#)
- [Alcohol and Drug Intoxication and Withdrawal Management Clinical Guideline](#)
- [Sexual Safety Policy](#)
- [Sexual Assault: Responding to Allegations Policy](#)
- [LGBTIQ+ Inclusivity for Patients Policy](#)
- [Prescribing Policy](#)
- [Consumer, Family and Carer Information SOP \(MH\)](#)
- [Special and Supportive Observations Policy](#)
- [Potentially Harmful Items Management for Mental Health Patients SOP](#)
- [Patient Property Management SOP](#)
- [Progressive Risk Assessment SOP](#)
- [Minimisation and Safe Management of Restraint and Seclusion \(for Mental Health Areas\) CPS](#)
- [Security: Request for External Security Staff SOP](#)
- [Personal Electronic Communication Devices for MH Inpatient Consumers Policy](#)

EMHS Policy Hub

- [Security Policy](#)
- [Discharge Policy](#)
- [Discharge Communications Policy](#)
- [Discharge Against Medical Advice Policy](#)
- [Transportation of Mental Health Consumers Policy](#)
- [Supporting Transition from Hospital \(7 day follow up\) Guideline](#)
- [Notifications Requirements under the WA Mental Health Act 2014 Policy](#)
- [Missing or Suspected Missing Mental Health Patients/Consumers Policy](#)

WA Department of Health Policy Hub

- [SSCD for Mental Health Services Procedure](#)
- [Consent to Treatment Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)
- [Mental Health Data Collection: Data Specifications \(July 2023\)](#)

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Related policy, practice standard, clinical guidelines cont'd

Other

- [AMHCC User Manual - IHACPA](#)
- [Australian Mental Health Outcomes and Classification Network MH NOCC Technical Specifications](#)
- [Chief Psychiatrist's Standards for Clinical Care](#) and [Charter of Mental Health Care Principles \(MHA 2014\)](#)
- [Policy for Reporting of Notifiable Incidents to the Chief Psychiatrist – Public Mental Health Services 2018](#)
- [Mental Health Act \(MHA\) 2014](#)
- [Carers Recognition Act 2004](#)
- [Guardianship and Administration Act 1990](#)

Related forms

- *Progressive Risk Assessment (PRA)*

Links to resources

- [Take 5: Personal Duress Alarms \(IOTTAG GPS\)](#)
- [Take 5: WebPSOLIS](#)

Legislative requirements, the evidence, and the standard

For information relating to the legislative requirements and standards that RPBG policy documents must adhere to, and regarding the logos and levels of evidence used within RPBG policy documents, refer to [Legislative Requirements, the Evidence and the Standard](#) (live link) on the Policy Hub.

Related national standards

[ACSQHC NSQHS Standards 2nd Edition \(2021\)](#): Standard 1: Clinical Governance; Standard 2: Partnering with patients; Standard 5: Comprehensive Care; Standard 6: Communicating for Safety; Standard 8 – Recognising and Responding to Acute Deterioration

[NSMHS Standards \(2010\)](#)

Standard 1: Rights and responsibilities; Standard 2: Safety; Standard 3: Patient and carer participation; Standard 6: Patients; Standard 7: Carers; Standard 8: Governance, leadership, and management; Standard 10: Delivery of care.

References

1. Government of Western Australia. 2014. *Mental Health Act 2014 (WA)*. WALW - Mental Health Act 2014 - Home Page (legislation.wa.gov.au)
2. Australian Government. (2013). *A National Framework for Recovery-Oriented Mental Health Services: policy and theory*. Department of Health and Aged Care. [Online]

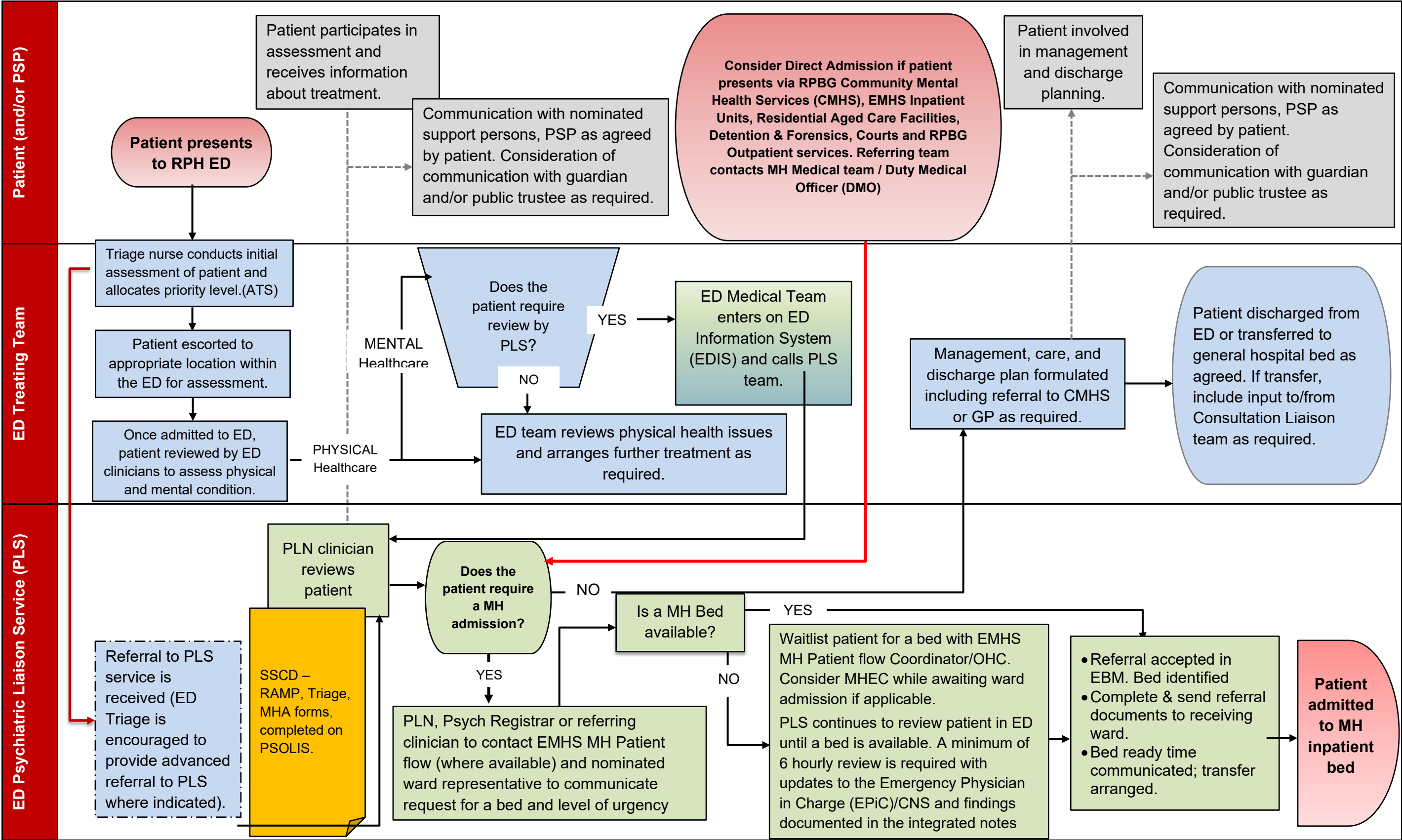
Mental Health Inpatient Triage to Discharge SOP

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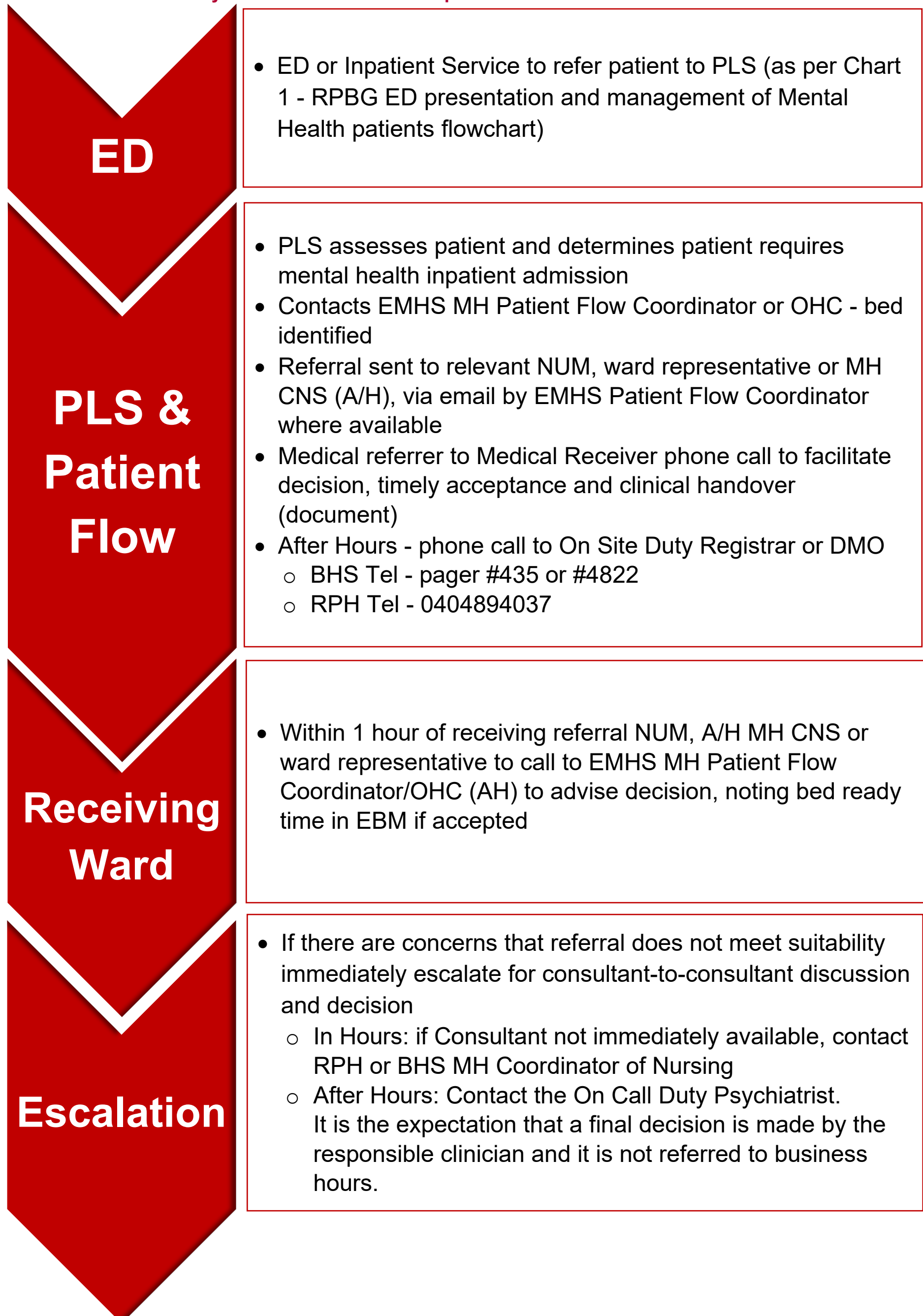
Appendix I: Mental Health (MH) Direct and Emergency Presentations to RPBG

Chart 1 – RPBG presentation and management of Mental Health patients



Appendix I cont'd

Chart 2 – Referral Pathway from RPH ED to RPBG Inpatient Mental Health Services



Appendix II: Specific ward/unit requirements

EMyU - BHS	Wards 6 & 7 - BHS	Mental Health Unit (MHU) - RPH
Restricting access to courtyards • EMyU & Ward 6: Patients must be supervised at all times during the following courtyard access times: 09:00-19:00 hours, except during patient mealtimes and staff handover. For courtyard access during 22:00-06:00 hours, obtain approval by the Shift Coordinator / NUM • Ensure visits are completed in visitor rooms only. No visitor access to courtyards, other areas/bedrooms • Ward 7 - courtyard is open 06:00-22:00hrs with unrestricted access for patients. For courtyard access outside of this time, obtain approval by the BHS MH A/H CNS. Visits preferentially occur in the room connected to the dining space or the activity room (rarely in the pool room). No visitor access to courtyards, other areas/bedrooms. Restricting access to meals or activities • Ensure safe-type cutlery are used for meals and activities of daily living.		Prior to RPH transfer, MHU allocated clinician or Treating Team to: • Advise the referring clinician to enter MHU using secure lobby entry unless alternative entry is necessary. Referrer to use the intercom to contact MHU Team • Liaise with PSS, to confirm transfer, expected starting time and ensure a minimum of 2 security officers (contact RPBG Security HealthPoint page) or more attend the transfer to ensure staff safety, given the area lacks duress capabilities • Meet PSS and Security staff at drop-off area at the earliest opportunity using the Voluntary Transport route • A/H and/or weekends, walk to Voluntary Transport area (A block, level 2) to assess area is suitable and liaise with referring team • Facilitate PSS staff to act as forward scout to minimise public traffic, clear transport routes and remove/ minimise hazards, if required • Convey transport team maintain patient responsibility until accepted into MHU where applicable. MHU Shift Coordinator to: • Inform Ward 2K Shift Coordinator of pending admission and advise staff access between the MHU and Ward 2K is suspended until patient is accepted onto MHU. MHU Team to escort patient with Security staff to secure lobby entry area or an interview room.
EMyU • Banksia Hill Detention Centre (BHDC) Referral or Hospital court order for assessment and treatment ○ BHDC patients must be admitted into the secure Psychiatric Intensive Care Unit (PICU). Open access will be considered only after consultant review and if deemed appropriate ○ 2 x BHDC prison guards are to be present with each BHDC patient, in situ 24/7. Refer to EMyU: Youth Custodial Patient Management SOP. • Accepts admission of patients with an eating disorder - admissions capped at 2 patients at a time.	Ward 6 Escort • During escort or transfers of patients, determine if necessary to inform switchboard and request the front door to F block reception is locked while transfer occurs • Use the airlock entry during the transfer of the patient • Patients should not be escorted via the volleyball court due to safety risks. Ward 7 - Access to escorted ground access (EGA) and unescorted ground access (UGA) is as following: 08:30-12:30 hours / 14:30-18:00 hours	

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Appendix III: MHEC Admission

The following admission inclusion criteria and MHEC Traffic Light System (TLS) guide the selection and prioritisation of suitable patients referred for admission to the MHEC.

Patients:

- Who are voluntary and aged 18 years and above
- Presenting with signs and symptoms indicating a mental illness which likely requires acute short stay inpatient care
- Assessed as medically suitable for admission to a mental health ward by referring clinician
- Who are aware of, and consent to referral

Note: Case-by-case considerations for admissions apply to:

- Patients under the **MHA 2014** once risk assessed and deemed to be low risk of aggression/ behavioural disturbances as per MHEC Consultant, or delegate.

MHEC Traffic Light System, RPH

Suitable for MHEC	• Additionally, to referral criteria, MHEC TLS guides the selection and prioritisation of suitable patients from the RPH ED and RPBG CMHS • Aged 18 years and above • With signs and symptoms that indicate a mental illness which likely requires acute short stay (typically up to 72 hrs) inpatient care. Note: Allow flexibility when there is a Code Yellow or Code Brown following consultation with consultants and advice from Mental Health Divisional Medical Co-Director or After Hours delegate • Reporting self-harm/suicide ideation not requiring immediate medical treatment and who can guarantee their safety in the immediate future • Who are medically assessed as suitable for admission to a mental health unit • Who require admission to a MH unit other than MHEC, have a transfer plan and are safe to wait in the MHEC instead of ED • Who are aware of and consent to referral.
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Appendix III: MHEC Admission - MHEC Traffic Light System, RPH cont'd

For Consideration to MHEC	• Presenting with drug/alcohol issues with underlying mental illness • Presenting with social issues, including homelessness with underlying mental illness • With intellectual impairment with underlying mental illness • Requiring intravenous (IV) infusion with underlying mental illness • Referred by EMHS CMHS • Under the MHA 2014 and presenting with low - moderate aggression as per MHEC Consultant, or delegate • Who are detained under the MHA 2014 on Ward 2K • Who have insight / are compliant • Presenting with depressive illness • Presenting with suicidality or drug-induced psychosis • With alerts for violence or aggression (low – moderate).
Not Suitable for MHEC	• Requiring immediate IV sedation • With complex and/or unstable medical problems, requiring admission to a general hospital bed • Aged under 16 years • Referred by other EDs or out of catchment CMHS • Who do not have a mental illness • Who are likely to require a long-term MH inpatient admission • Assessed as high risk relating to behavioural disturbances or aggression • Presenting with primary drug and/or alcohol intoxication or withdrawal without an underlying mental illness.

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Appendix IV: Specific Ward 2K requirements (RPH)

- Ward 2K is an unauthorised treatment area; the intention and function of the ward is for voluntary patients and not for the assessment and management of patients placed under the **MHA 2014**, however, in the absence of a secure MH bed, these patients may be considered for admission on a case-by-case basis via consultation with the Ward 2K Consultant, or Medical Co-Director, or Duty Psychiatrist.
- Additionally, to referral criteria, Ward 2K Traffic Light System guides the suitability and prioritisation of all patients to Ward 2K

Ward 2K Traffic Light System	
Suitable for Ward 2K	• Presenting with signs and symptoms which indicate a mental illness • With no history of and assessed as low risk relating to behavioural disturbances and/or aggression • Assessed as compliant with treatment • Assessed as low risk relating to self-harm or suicidal ideation • Referred by external EDs/MH wards
For Consideration for Ward 2K	• Detained under the MHA 2014 • Who are minors • On <i>Form 6B</i> (Inpatient treatment order in a general hospital) • With no history of chronic risk of harm to self/others and with no imminent plan or intent • Responding to medical treatment and intervention • Referred by EMHS CMHS • Posing with the following micro alerts: ○ Methicillin-Resistant Staphylococcus Aureus (MRSA): Micro Alert B, C and/or W on webPAS as per the criteria in the RPBG MRSA - Ward Management Guidelines ○ Multi Resistant Gram Negative Bacteria (MRGNB): Micro Alert Y ○ Vancomycin-resistant Enterococci (VRE): Micro Alert F and continent
Not Suitable for Ward 2K	• At imminent high risk of harm to self or others • Presenting as highly psychotic with a recent history of aggression • Who have not responded to medical treatment, or have required mechanical restraint • Remaining in a highly aroused (mental or physical) state which has not diminished significantly since presentation and initial assessment • Posing with the following micro alerts: ○ CPE: Micro Alert G, H ○ C auris: Micro Alert J, K ○ VRE colonised consumer; Micro Alert V, VRE contact: Micro Alert F and incontinent

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Appendix V: Ward 10 (BHS) - Older Adult (OA) specific requirements

Inclusion criteria:

- Aged – 65 years or older (55 years for Indigenous persons who self-identify as OA)
- Reside in the EMHS catchment area and demonstrate
- Dementia diagnosis with behaviour disturbance (can include persons younger than 65yrs when these symptoms present)
- Late onset mental illness or persons not active with mental health services for 2 years or more
- Mental illness symptoms and whose needs are best met in an OA care setting
- Completion of organic screen, including FBC, U & Es, B12, LFT, Random BSL, Folate, TFT, MSU, Calcium, drug levels (lithium or Sodium Valproate)

Exclusion criteria

- Intellectual disability
- Acute physical illness
- Forensic assessments or Expert opinions to courts

Other specific requirements

- Consider and request any reports from aged care facility and referrer to provide information on strategies trialled, or, if Dementia Support Australia involvement for OA patients with age-related functional decline or diagnosed age-related co-morbidity
- For all patients complete:
 - *MR32.90 Mini Mental State Examination (MMSE)*
 - *MR150.10 Bowel Chart*
 - *MR00H.1 Goals of Patient Care* and/or electronic print and file in patient's medical record.

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Appendix VI: Transitional Care Unit (TCU) specific requirements (offsite)

- **TCU admission prioritisation - Referral from EMHS inpatient areas and CMHS**
[Bidi Wungen Kaat Centre \(TCU\) PRU /RRU Models of Care](#) is based on utilising Peer Carer Recovery Workers (PCRW) as part of the care team.
- Complete patient's identification via use of *Photo Identification Consent Form*
- **What Matters Most Record (WMMR)**
 - PCRW to complete plans: MINDS My Better Life Plan (Recovery Plan) and Client Wellness Plan
 - A copy of which is reviewed weekly and filed in the patient medical record. All documents inform the TSDP in PSOLIS and WMMR
- **PRA completion**
 - Only required at the following circumstances:
 - Certain instances of leave as below
 - When a new risk factor is identified (i.e., self-harm, infection)
 - Post incident (i.e., fall, self-harm)
 - On condition change
- **Leave management** – e.g., when a patient has taken leave or had leave approved in excess of 2 hours or day leave including weekends, e.g. off the hospital or service grounds complete *PRA* and place in the patient healthcare record
 - Discuss leave responsibilities with the patient, including provision of the TCU contact number for patient to call if issues arise or to advise staff if they may be later to return than expected. If this is not the case further interventions will be put in place. If there are any issues during leave, staff are to contact the patient. If not contactable and concerns are present, contact NoK or WAPOL, if deemed necessary. Refer to [Escalation of care](#) section and [EMHS Missing or Suspected Missing Mental Health Patients/Consumers](#)
- **Intentional rounding** – 2 hourly rounding. Some patients can request to be 'exempt' from nocte observations between 21:00 and 07:00hrs if approved by MO on the basis of completed/updated risk assessment and documented in the medical record. This can change if the risk changes.
- **Performing vital signs** - at time of TCU admission or initial assessment: as a minimum, perform baseline set of vital signs - temperature, pulse respirations (TPR), blood pressure (BP), oxygen saturation (SpO₂)
 - **Rehabilitation and Recovery Unit (RRU):**
 - Thereafter, complete vital signs as clinically indicated or as requested by MO
 - **Prevention and Recovery Unit (PRU):**
 - Thereafter, staff to check-in with patient for self-reported changes in their condition when administering medications and when intentionally rounding. Repeat vital signs if abnormal signs or symptoms emerge or reported changes
 - Note: the level of monitoring expected in the PRU is the same as would normally occur in the community or via GP monitoring
- Document findings on *TCU Adult Observation and Response Chart (AORC)*
- Exceptions to minimum baseline vital signs monitoring would be when indicated by other, relevant medication policy guidelines (i.e. Clozapine)

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Appendix VI: Transitional Care Unit (TCU) specific requirements (offsite) cont'd

Escalation of care

- For emergencies that require escalation of care beyond the local response staff are to call 000 (not 55 as per other inpatient areas). Refer to [Emergency Procedures- Transitional Care Unit](#) and further guidance regarding Codes Black, Yellow, Red, Purple, Orange and Brown
Also refer to **Appendix VI (TCU Escalation of care) in the [RRAD in Mental State, Cognition and Behaviour SOP](#)**
- **Medication dispensing, supply, and storage.** Refer to the specific agreement as per the [Medication Management and Storage Clinical Guideline](#).
Processes for the refrigeration of medications or stored in the medication room or cleaning requirements, as per the [Medication Refrigerators Safe Use SOP \(BHS & Midland\)](#).
- **TCU facilities management** - Sign in, key and facilities management completed as per the [EMHS Bidi Wungen Kaat Centre Defect Management and Maintenance Access Procedures](#) and [RPBG Access to Clinical Areas SOP](#).

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Appendix VII: MH Division Inpatient Ward Security Checklist

Complete all details at commencement of shift (minimum) and more frequently as required

Ward/Unit _____

Area to be checked (initial each shift)	Date					
Relevant ward/unit doors locked						
Restricted items returned/signed back as appropriate						
Additional areas specific to ward/unit area						
Concerns escalated to NUM/MH CNS						
Print Name						
Signature						
Time						

Area to be checked (initial each shift)	Date					
Relevant ward/unit doors locked						
Restricted items returned/signed back as appropriate						
Additional areas specific to ward/unit area						
Concerns escalated to NUM/MH CNS						
Print Name						
Signature						
Time						

This checklist can be obtained from NUMs/CNSs secure storage area

Place completed checklist in identified secure storage area

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